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MEDBRIEF SYNTHETIC MEDICAL REPORT (TEST DATA ONLY - NOT A REAL PATIENT)

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PATIENT INFORMATION:

Patient Name: John Doe (Synthetic)
Age: 54 | Gender: Male | DOB: 1970-05-15
MRN: SYN-987654 | Date of Visit: 2026-09-10
Attending Physician: Dr. Jane Smith, MD (Cardiology)

CHIEF COMPLAINT:

Patient reports shortness of breath on exertion and recurrent mild chest tightness over the past two weeks.

CLINICAL HISTORY & PRESENT ILLNESS:

The patient is a 54-year-old male with a history of essential hypertension and hyperlipidemia. He reports experiencing progressive exertio

PHYSICAL EXAMINATION:

- Vital Signs: BP 138/86 mmHg, HR 76 bpm, RR 18/min, Temp 98.4°F, SpO2 97% on room air.
- Cardiovascular: Regular rate and rhythm. S1 and S2 present. No S3, S4, or murmurs.
- Respiratory: Clear to auscultation bilaterally. No wheezes, rales, or rhonchi.
- Extremities: No peripheral edema. Pulses intact 2+ bilaterally.

DIAGNOSTIC WORKUP & LAB RESULTS:

- Electrocardiogram (ECG): Normal sinus rhythm at 74 bpm. No acute ST-T wave changes.
- Troponin I: < 0.01 ng/mL (Normal)
- BNP: 45 pg/mL (Normal)
- Lipid Panel: Total Cholesterol 215 mg/dL, LDL 135 mg/dL, HDL 42 mg/dL, Triglycerides 190 mg/dL.
- Echocardiogram: Left ventricular ejection fraction (LVEF) 60%. Mild left ventricular hypertrophy. No regional wall motion abnormalities.

IMPRESSION / DIAGNOSIS:

1. Exertional dyspnea secondary to mild hypertensive heart disease and hyperlipidemia.
2. Acute coronary syndrome ruled out based on serial cardiac biomarkers and ECG.

TREATMENT PLAN & RECOMMENDATIONS:

1. Continue Lisinopril 10 mg orally once daily for blood pressure management.
2. Initiate Atorvastatin 20 mg orally once daily for hyperlipidemia.
3. Schedule an outpatient treadmill stress test within 2 weeks.
4. Dietary modifications: Low-sodium, heart-healthy Mediterranean diet.
5. Follow-up clinic appointment in 4 weeks or sooner if symptoms worsen.